Acute Asthma Exacerbation

- Check → **BP, HR, RR, O2, PEFR** (if available)
- Assess → **Severity (Moderate, Severe, Life threatening, Near Fatal)**

Give:

- **O2 driven Nebulization if hypoxemia** (Target SpO2 94-98%)
- **Steroid** in adequate doses to all patients with an acute asthma attack (Oral preferred to IV/IM)
- Continue Prednisolone **Adult: 40-60mg daily** or **Pediatrics: 1 mg/kg/day** until recovery (5-7 days)
- Assess response → **Repeat Ventolin every 20 min for 1 hour if indicated**
- If poor response → **Refer to medical casualty by ambulance**

Medication	Adult dose	Pediatric dose	Age
Salbutamol (Ventolin Inhaler with spacer OR Nebulizer if hypoxemia)	Inhaler (MDI + spacer): 4-10 puffs	Inhaler (MDI + spacer): 2-6 puffs 2-10 puffs	Infants and children < 5 y Children > 6 y
	Nebulizer: 5mg = 1 ml	Nebulizer: 2.5mg = 0.5ml	
Ipratropium bromide (Atrovent Nebulizer) → if severe or no response	0.5mg = 2 ampules	0.25mg = 1 ampule	

Medication	Adult dose	Pediatric dose	Age
Oral prednisolone	40-60 mg	(1 mg/kg/day) 20 mg/d 30-40 mg/d	2-5 years > 5 years
Or IV/IM Hydrocortisone (off-label dose)	100 mg	50mg 100 mg	<2 years 2-5 years